BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show order, not equal periods. These are expected checks, not booked appointments. Record the booking owner, due window, result reviewer and escalation contact.

Check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Bloods / symptoms	FBC, renal and liver profile	FBC, renal and liver before each cycle	Review tolerance, symptoms and results	Repeat checks before treatment; extra tests if needed	Follow abnormalities; agree next care
ECG / electrolytes	Baseline ECG required	Regular ECG; confirm timing	Review symptoms and new medicines	Regular ECG	Agree follow-up if abnormal
Echo / MUGA	Baseline echo / MUGA required	Repeat if clinically indicated	Check cumulative anthracycline exposure	Echo before each further administration beyond cumulative 450 mg/m²	Exposure- and risk-based cardiac follow-up
Imaging / markers	Baseline disease assessment	Earlier review for new symptoms	Agree scan type and due window	Breast: generally every 2–4 months; other cancers: specific plan	Review scans with symptoms; markers are supplementary
Bone health / DEXA	Assess endocrine / fracture risks	No automatic cycle-based DEXA	DEXA if indicated	Repeat according to bone-risk plan	Agree ongoing bone care

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00205 v8 · official source · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026 · EMA Caelyx pegylated liposomal product information

THE WHOLE TREATMENT JOURNEY

Pegylated liposomal doxorubicin · every 4 weeks: the treatment pathway

Source-defined advanced breast, ovarian, endometrial cancer or soft-tissue sarcoma

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First cycle / visits	Early reassessment	Later cycles	Change / completion
Cancer medicine NCCP 00205	Confirm exact indication / formulation	Treat on Days 1 of 28	Review tolerance and benefit	Continue / adjust / pause after review	Agree next treatment or care
Surgery / radiotherapy METASTATIC	Define any local-treatment goal	No routine concurrent radiation assumed	MDT agrees whether to sequence or overlap	Agree any chemotherapy pause with both teams	Reassess symptom / local-control needs
Bloods / ECG NCCP 00205 · SMPC	Baseline bloods / cardiac assessment	Bloods before each cycle	Regular ECG	Review results before treatment	Follow abnormalities
Echo / MUGA NCCP 00205 · SMPC	Echo / MUGA required	Earlier test for cardiac symptoms	Review exposure / cardiac risk	Echo before each further administration beyond 450 mg/m² cumulative	Agree cardiac follow-up
Imaging / markers NCCP 00205 · METASTATIC · MARKERS	Baseline disease assessment	Symptoms may bring scans forward	At the agreed response milestone	Review scans with symptoms / markers	Document benefit and next plan
Bone / additional care BONE	Check fracture and endocrine risks	No automatic maintenance phase	DEXA if indicated	Supportive and palliative care as needed	Agree ongoing care

What each check is for

1 **Bloods**

FBC, renal and liver profile before starting and before each 28-day cycle. Check mouth soreness, palms / soles and recovery from earlier toxicity before each dose.

Results and symptoms guide treatment safety.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00205

2 **Heart checks**

ECG is part of baseline and regular monitoring. The NCCP source advises frequent ECG monitoring; the team should specify when it is due rather than assuming an arbitrary cycle interval. Baseline echo / MUGA is required. Repeat during treatment as clinically indicated, and before each additional administration that exceeds a lifetime cumulative anthracycline exposure of 450 mg/m². Prior anthracyclines count; this threshold is not an automatic treatment stop.

ECG and echo answer different questions.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00205 · SMPC

3 **Response review**

For metastatic breast cancer, ESMO recommends response evaluation generally every 2–4 months, adjusted to disease, treatment and symptoms. Agree the scan type, due window and result-review appointment; symptoms can prompt earlier assessment. For the other cancer indications on this card, use the cancer-specific monitoring plan; the breast interval is not transferred automatically.

Review benefit before continuing treatment.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00205 · METASTATIC

4 **Handover**

At initiation, each review and treatment change

Name the booking owner, due window, reviewer and escalation contact.

Ask / act: Confirm who arranges the check and who reviews the result.

Sources: NCCP 00205

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.